

OET SPEAKING

COMPREHENSIVE RULEBOOK

AI Training Edition — Complete Rules & Communication Guidelines
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Rulebook Type	OET Speaking Sub-test — Complete Guidelines
Target Users	OET Candidates / AI Training Datasets
Source	Live Arabic-language Teaching Sessions (2 videos)
Language	English (extracted & structured from Arabic instruction)
Content	Communication Rules · Language Rules · Clinical Rules

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INTRODUCTION & HOW TO USE THIS RULEBOOK

This rulebook compiles all communication rules, language conventions, and clinical interaction guidelines taught across two OET Speaking preparation sessions. It is structured for OET candidates and as a training dataset for AI-powered OET preparation tools. Each rule is numbered sequentially and categorised by domain.

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SECTION 1 — OET SPEAKING EXAM FORMAT & STRUCTURE

RULE 01 Two-Card Exam Structure

- The OET Speaking sub-test consists of exactly two role-play cards: Card 1 and Card 2.
- Each card: 3 minutes preparation + 5 minutes discussion = 8 minutes per card.
- Total exam duration: approximately 20–22 minutes.
- Warm-up questions precede the cards and are NOT counted in the final score.

■ Use warm-up questions to build confidence — they are a free opportunity to settle your nerves.

RULE 02 Recording, Not Video

- The exam is audio-recorded only — there is no video camera.
- Your score is determined by the recording, NOT by the interlocutor in front of you.
- Eye contact, hand gestures, and facial expressions are NOT assessed.
- The interlocutor is an ACTOR playing the patient — they are not your examiner.

■ You are free to keep your eyes on the card during the discussion.

RULE 03 Paper vs. Computer-Based Exam Schedule

- Paper-based: Speaking on the same day as Listening and Reading.
- Computer-based: Speaking is usually scheduled on a different day.

RULE 04 Required Grade

- Passing grade: Grade B (approximately 350 on the OET scale).
- Warm-up questions do not count toward the grade.

RULE 05 Card Tasks — Obligatory vs. Optional

- The tasks listed on the card are NOT obligatory — they are guides, not scripts.
- You may speak about topics not listed in the tasks.
- Task order is NOT obligatory — follow the patient's lead.
- If the patient asks out of order, answer them, then return to your task sequence.

■ NEVER treat the card as a lecture script. The conversation must be a 'ping-pong' dialogue.

SECTION 2 — LANGUAGE RULES: MEDICAL JARGON vs. LAYMAN'S LANGUAGE

Never use medical jargon, abbreviations, or technical terminology when speaking directly to a patient. Always replace medical terms with their everyday English equivalents.

RULE 06 Core Language Rule

- Never use medical jargon or abbreviations with patients.
- Replace every technical term with its plain English equivalent.
- Two approaches: (a) Use layman's term only, or (b) Use medical term + ask for feedback + explain.

■ *Instead of 'CT scan', say 'imaging scan'.*

■ *Instead of 'endoscopy', say 'camera test'.*

■ **Do NOT use:** CT scan, MRI, ultrasound, endoscopy, colonoscopy, or any -scopy word directly.

RULE 07 Imaging Investigations

- CT scan → 'imaging scan'
- MRI scan → 'imaging scan'
- Ultrasound → 'gel scan' or 'tummy scan'
- X-ray → 'X-ray' (acceptable as-is)
- All endoscopy types → 'camera test'

■ *'We will arrange an imaging scan of your chest.'*

■ *'I'd like to arrange a camera test to look inside your stomach.'*

RULE 08 Injections

- IV/IM injection → 'a jab'
- Subcutaneous injection → 'a tiny injection just under the skin'
- Spinal injection → 'a tiny injection in your back'

RULE 09 Monitoring & Investigations

- Blood pressure (BP) → 'measure the pressure in your blood vessels'
- ECG → 'a tracing of your heartbeat'
- EEG → 'measuring the electrical activity in your brain'

RULE 10 Body Parts

- Abdomen → 'tummy'
- Oesophagus → 'food pipe'
- Trachea → 'windpipe'
- Uterus → 'womb'

RULE 11 Medical Specialists

- Paediatrician → 'children's specialist'
- Gynaecologist → 'women's health specialist'
- Endocrinologist → 'hormone specialist'
- General principle: [specialty] + 'specialist'

RULE 12 When to Use the Medical Term — Feedback Approach

- If you use a medical term, immediately ask for feedback.
- Ask: 'I'm sorry for the medical term — have you heard of [condition] before?'
- Then explain the term in plain English regardless of the answer.

■ **The safest approach is to use layman's language directly and avoid medical terms altogether.**

QUICK REFERENCE: Medical Term → Layman's Equivalent

Medical Term	Plain English	Example Phrase
CT scan / MRI scan	Imaging scan	'We'll do an imaging scan of your chest'
Ultrasound	Gel scan / Tummy scan	'A gel scan of your abdomen'
X-ray	X-ray (unchanged)	'We'll take an X-ray of your leg'
Endoscopy / Colonoscopy	Camera test	'A camera test to look inside'
IV / IM Injection	A jab	'A quick jab in your arm'
Subcutaneous injection	Tiny injection under the skin	'A small injection under your skin'
ECG	Tracing of your heartbeat	'A tracing to check your heartbeat'
EEG	Measuring brain activity	'Measuring electrical activity in your brain'
Blood pressure	Pressure in your blood vessels	'Let me check the pressure in your vessels'
Abdomen	Tummy	'Any pain in your tummy?'
Oesophagus	Food pipe	'Irritation in your food pipe'
Trachea	Windpipe	'Swelling in your windpipe'
Uterus	Womb	'A scan of your womb'
Paediatrician	Children's specialist	'Refer to a children's specialist'
Gynaecologist	Women's health specialist	'A women's health specialist can help'
Endocrinologist	Hormone specialist	'A hormone specialist would be best'
Hypertension	High blood pressure / higher side	'Your reading is on the higher side'

SECTION 3 — COMMUNICATION & CONSULTATION STRUCTURE

RULE 13 Opening — Greeting and Introduction

- Begin with a professional greeting.
- State your name and role, invite the patient to sit.
- Ask an open-ended opening question.

■ *'Hello, Mr. Jones. I'm Dr. Smith. Nice to meet you. Please take a seat. How can I help you today?'*

■ *Second visit: 'Hello, nice to see you again. As a reminder, I'm Dr. Smith — I looked after you for [condition].'*

RULE 14 Active Listening During Patient History

- Allow the patient to tell their story without interruption.
- Use active listening signals: 'I see', 'Okay', 'Right', 'Mm-hmm'.
- Do NOT speak over the patient or rush their story.

RULE 15 Empathy After Patient Story

- After the patient finishes, ALWAYS express empathy before moving forward.
- Acknowledge what the patient has been through.

■ *'I'm really sorry to hear that. That must have been very difficult for you.'*

RULE 16 Asking Permission Before Questions

- Always ask the patient's permission before proceeding with more questions.

■ *'Would you mind if I asked you some more questions about your condition?'*

RULE 17 Establishing the Diagnosis

- Check the patient's prior knowledge before delivering a diagnosis.
- Present in plain language — never cause unnecessary alarm.

■ *'Based on what you've told me, it seems you may have a condition called GORD — have you heard of it before?'*

RULE 18 Taking Feedback After Diagnosis

- After explaining, ALWAYS take feedback from the patient.
- This is a core OET communication requirement.

■ *'Does that make sense to you?'*

■ *'Is there anything you'd like me to clarify?'*

RULE 19 Explaining Causes

- Explain the cause(s) in plain language.
- Link known risk factors from the patient's history to the condition.

RULE 20 Recap / Summarising the Card

- Briefly recap everything discussed at the end of the card.
- Keep it to approximately 30 seconds.

■ *'So, to recap: you have [condition], we've talked about [changes], and I've prescribed [medication]. Any questions?'*

RULE 21 Closing the Card

- End with a standard, professional closing phrase.
- Offer ongoing support and wish the patient well.

■ *'I wish you all the best. Please don't hesitate to come back if you have any concerns.'*

RULE 22 Ping-Pong Conversation Rule

- The conversation MUST be a two-way dialogue — never a monologue.
- Speak, then let the patient respond. Every point should invite a reply.
- Continuous speaking without patient input is penalised.

■■ **Speaking for extended periods without allowing patient responses loses marks.**

SECTION 4 — LIFESTYLE COUNSELLING RULES

RULE 23 Weight — Asking Sensitively

- NEVER ask 'What is your weight?' directly — blunt and disrespectful.
- Approach the topic sensitively and indirectly.

■ *'Have you ever had any concerns about your weight?'*

■ *'Would you mind if I noted your height and weight to check your BMI?'*

■■ **Asking weight directly is a communication failure in OET.**

RULE 24 Exercise — Asking and Advising

- Ask about exercise habits before giving advice.
- Suggest specific, achievable targets.

■ *'Do you do any regular exercise at the moment?'*

■ *'Even a 30-minute walk each day can make a big difference.'*

RULE 25 Diet — Asking and Advising

- Ask about dietary habits using an open question.
- Advise on specific dietary changes relevant to their condition.

■ *'How would you describe your diet? Do you feel you eat healthily?'*

■ *'I'd strongly advise reducing your intake of fatty and processed foods.'*

RULE 26 Smoking — Initial Assessment

- Always ask whether the patient smokes before giving advice.
- If yes, explore: what, how many per day, how long.

■ *'Do you smoke at all?'*

■ *'How many cigarettes do you smoke per day?'*

RULE 27 Smoking — Negotiation (Black Area to Grey Zone)

- Use graduated negotiation — always start from the strongest recommendation.
- Step 1 (Black Area): Recommend complete cessation.
- Step 2 (if refused): Offer NRT alternatives.
- Step 3 (Grey Zone): Negotiate a reduction.
- NEVER start with Step 3 — you lose all negotiating leverage.

■ ■ *If you start with 'reduce smoking' and they refuse, there is nowhere left to go.*

RULE 28 Smoking — Empathy During Negotiation

- Acknowledge that quitting is genuinely difficult.
- Show empathy while maintaining your clinical recommendation.

■ *'I understand it's not easy, and I know many people find it very challenging to quit.'*

RULE 29 Smoking — Mention Cessation Support

- Always mention the smoking cessation clinic as a concrete resource.

■ *'We have a smoking cessation clinic here that could provide you with great support.'*

RULE 30 Alcohol — Assessment and Advice

- Ask about frequency and quantity (units per week).
- No 'alternatives' for alcohol — options are cessation or reduction only.
- Refer to an alcohol support service if appropriate.

RULE 31 Patient Autonomy — Universal Rule

- Always respect patient autonomy — you advise, not command.
- State this clearly if the patient is resistant.

■ *'No one can force you to do anything. I can only advise you — the decision is entirely yours.'*

RULE 32 High BP / Blood Sugar — Do Not Over-Diagnose

- Do NOT diagnose hypertension from a single reading.
- Inform calmly and ask about prior history.
- Transition naturally into lifestyle advice.

■ *'Your reading today is a little on the higher side. Have you ever been told your blood pressure was high before?'*

■ ■ *Saying 'You have hypertension' from one reading is clinically wrong and costs marks.*

RULE 33 Reassurance After Investigation Results

- When results are not serious, always reassure the patient.
- Acknowledge concerns and reframe positively.

RULE 34 Providing Specific Dietary Recommendations

- Advise against: fast food, fatty foods, processed foods, excess red meat.
- Advise for: vegetables, fibre, whole grains, lean proteins.

SECTION 5 — SECOND VISIT (FOLLOW-UP) CARDS

RULE 35 Identifying a Second Visit Card

- Look for: 'coming back for...', 'returning for...', 'follow-up', 'review of...'
- Test result cards are the most common second-visit type.
- 'Your patient...' alone does NOT confirm it is a second visit.

■ ■ *Do NOT assume every 'your patient' card is a second visit — look for explicit indicators.*

RULE 36 Greeting on a Second Visit

- Use a modified greeting acknowledging the previous encounter.

- A brief reminder of who you are is sufficient — skip the full introduction.

■ *'Hello, it's lovely to see you again, Mr. Jones.'*

■ *'Hello, good to have you back. As a reminder, I'm Dr. Smith — I treated you for [condition].'*

RULE 37 Test Result Cards

- Most common second-visit card: patient returns to hear investigation results.
- Before revealing results, check the support system.
- Deliver and explain results clearly.

RULE 38 Review of Medication Cards

- Ask about: effectiveness, side effects, compliance, and concerns.
- Adjust or continue medication based on feedback.

RULE 39 Review of Disease Cards

- Check on symptom changes, new symptoms, and medication adherence.
- Provide updated management plans as needed.

SECTION 6 — BREAKING BAD NEWS (CANCER & SERIOUS DIAGNOSIS CARDS)

RULE 40 Tone of Voice — The Single Most Important Element

- Your tone must be soft, low, and genuinely empathetic.
- Since there is no video, tone of voice is the PRIMARY communication tool.

■ *Speaking about cancer in a neutral or upbeat tone will cause very poor marks.*

RULE 41 Step 1 — Ask About Support System

- Before delivering any results, ask if the patient would like someone with them.

■ *'Before we discuss your results, is there anyone you'd like to have here with you?'*

RULE 42 Step 2 — Warning Shots

- Do NOT deliver a serious diagnosis suddenly — build up gradually.
- Give warning shots that prepare the patient emotionally.

■ *'I'm afraid the results are not quite what we had hoped for.'*

■ *'The results are showing something more serious than we expected...'*

RULE 43 Step 3 — Deliver the Diagnosis

- After warning shots, state the diagnosis directly but gently.
- Use plain language — never hide behind jargon.

■ *'I'm very sorry to tell you — the results are showing signs of cancer.'*

RULE 44 Step 4 — SILENCE (3–4 Seconds)

- After stating the diagnosis, STOP SPEAKING for 3–4 seconds.
- Allow the patient to absorb the shock.
- The patient may cry, go silent, or ask questions — all are normal.

■ *Continuing to speak immediately after delivering serious news without pausing is one of the most common exam mistakes.*

RULE 45 Step 5 — Respond to Emotional Reaction

- After the silence, acknowledge whatever reaction the patient has.
- Never dismiss or minimise their emotions.

■ *'I'm so sorry. I know this is a lot to take in. Please take all the time you need.'*

RULE 46 Step 6 — Give Hope and Next Steps

- After acknowledging emotions, provide forward-looking information.

- Mention early stage if applicable — use this as a positive frame.

■ *'I want you to know that we caught this at an early stage, and there are effective treatment options available.'*

RULE 47 Step 7 — End with Support System

- Return to the support system theme at the end.
- Ask who the patient has around them and offer your own support.

■ *'Is there someone who can be with you today?'*

■ *'I'm here for you, and my number is available whenever you need anything.'*

RULE 48 Proportionality

- Full protocol: cancer and serious diagnoses only.
- Minor bad news (fracture, mild result): simple acknowledgement and reassurance only.

■ *For a fracture: 'The X-ray shows a fracture. I know that's worrying, but the good news is it is manageable.'*

SECTION 7 — EXAM STRATEGIES & PERFORMANCE RULES

RULE 49 3-Minute Preparation Strategy

- Read the entire card from start to finish first.
- Underline key words in each task.
- Write a brief 1–2 word label next to each task.
- During the 5-minute discussion, glance at notes — don't re-read the full card.

RULE 50 Warm-up Questions — Preparation

- Typical questions: your name, where you work, why you're taking OET, specialty, career advice.
- Do NOT memorise scripts — adapt answers naturally to your own experience.
- Speak confidently — a strong start gives momentum.

RULE 51 Fluency — Reducing Hesitation

- Avoid filler words: 'umm', 'err', 'so...', 'like...'
- Practice compresses the translation process until hesitation disappears.
- Rehearse key phrases and card structures repeatedly.

RULE 52 Task Completion — Quality Over Quantity

- You do NOT need to complete all tasks to pass.
- A fluent, empathetic 3-task conversation scores better than a rushed 5-task performance.
- If you run out of time, close professionally.

RULE 53 Using Flexible Phrases

- Internalise ready-to-use phrases for: greetings, empathy, permission, diagnosis, advice, closure.
- Apply them naturally and adapt to each patient's situation.

RULE 54 Where to Look During the Exam

- Since eye contact is NOT assessed: keep eyes on the card throughout.
- Use your preparation notes to stay on track.
- This reduces cognitive load and helps fluency.

RULE 55 Differential Diagnosis in Writing

- Two acceptable formats: 'Possible diagnosis: X or Y' OR 'Differential diagnosis of X and Y'
- Both are correct — choose the one you are comfortable with.
- Do NOT write both formats together.

APPENDIX — CARD STRUCTURE AT A GLANCE

Stage	What To Do	Key Example Phrase
1. Greeting	Introduce yourself, invite to sit	'Hello, I'm Dr. X. Please take a seat.'
2. Opening	Open-ended question	'How can I help you today?'
3. Listening	Active listening signals	'I see... Right... Mm-hmm...'
4. Empathy	Acknowledge experience	'That must have been very difficult.'
5. Permission	Ask to proceed	'Would you mind if I asked some questions?'
6. Questions	Explore symptoms	'Can you tell me more about the pain?'
7. Diagnosis	State in plain language + feedback	'It seems you may have... Have you heard of this?'
8. Causes	Explain causes plainly	'One main cause is...'
9. Lifestyle	Counsel sensitively	'I'd strongly advise you to consider...'
10. Treatment	Explain + feedback	'I'd like to prescribe... How does that sound?'
11. Reassurance	Reduce anxiety	'I want to reassure you we can manage this well.'
12. Recap	30-second summary	'So to recap what we've discussed...'
13. Closure	Close warmly	'Please don't hesitate to come back anytime.'